

**SYNTHETIC COURT DOCUMENT — FICTIONAL CASE — NOT AN
ACTUAL FILING**

EMS Patient Care Report

Service: Alder Creek Fire and Medical **Patient:** Mara Ellison **Incident:** February 3, 2025 **Lead clinician:** Paramedic Mateo Cole **Civil exhibit lodged:** November 21, 2025

This fictional patient-care report documents assessment and transport after a police welfare encounter. It is a clinical record, not a use-of-force reconstruction. Statements attributed to Ellison or officers identify the source of a treatment history. Cole did not witness the critical physical contact and offers no opinion about whether Ellison struck an officer, reached toward equipment, or merely pulled away.

Dispatch assigned the medical unit at approximately 19:54 for a person exposed to one reported close-contact conducted-energy cycle. The location was the Building C breezeway at Larkspur Court Apartments. The dispatch note also mentioned wrist and shoulder discomfort. The crew arrived at approximately 20:01 and found Ellison seated upright with Officers Nolan Rusk and Priya Vega nearby.

Ellison was awake, breathing without assistance, and able to answer direct questions. She appeared tearful and distressed. No probe removal was required because officers reported drive-stun contact rather than fired darts. The scene was quiet by medical arrival, and no weapon was presented to the crew. Police described the event as a welfare contact with no criminal booking planned.

Scene history and source attribution

Ellison told Cole that an officer grabbed her left wrist, she pulled because it hurt, and she then felt the energy device while being forced down. She denied striking the officer or reaching for his belt. That account was recorded because it informed the locations examined; it was not independently verified by the medical team.

Rusk told the crew that Ellison twisted, hit his forearm, and reached toward duty equipment during restraint. He reported one approximately five-second drive-stun cycle and no probe deployment. Vega said she assisted with hand control but did not have a reliable view of Ellison's hands during the closest contact. These competing statements were not resolved at the scene.

Ellison complained principally of burning at two contact points, soreness and abrasion at the left wrist, and pain over the left shoulder with lifting the arm. She denied chest pain, shortness of breath, loss of consciousness, head strike symptoms, neck pain, numbness, or weakness. She asked that only one person speak at a time.

The crew confirmed that no second electronic cycle was reported. Cole requested that police reduce crowding during assessment. Ellison cooperated with vital signs and focused examination after the steps were explained. Her sister Iris remained outside the clinical work area until Ellison agreed that she could approach.

Primary assessment and vital observations

Airway was open, speech was clear, and respirations were unlabored. Radial pulses were present and regular. Skin was warm except for the exposed forearm in cool evening air. Ellison was oriented to person, place, approximate time, and the reason medical personnel were present. She followed one-step clinical directions when given separately.

Initial recorded observations were blood pressure 138/84, pulse 104, respirations 20, oxygen saturation 99 percent on room air, and temperature 37.0 degrees Celsius. The mildly elevated pulse decreased during reassessment. No dysrhythmia was apparent on the field rhythm strip. These observations did not rule in or rule out the mechanics of the preceding struggle.

The left hand was warm with prompt capillary refill. Ellison moved all fingers and reported normal sensation. A narrow superficial abrasion encircled part, but not all, of the left wrist. There was no gross deformity. Two small erythematous contact areas were present along the left lateral torso near the lower rib margin. Each was superficial, with intact surrounding skin and no active bleeding.

Ellison guarded her left shoulder and reported pain with active elevation. Cole found no obvious dislocation, clavicle deformity, or loss of distal circulation. Because field examination could not exclude a strain or occult injury, transport for further assessment was recommended.

Treatment and reassessment

Cole cleaned the wrist abrasion with sterile saline and placed a nonadherent dressing. The two contact areas were cooled briefly and covered with dry protective dressings consistent with superficial skin care. No embedded probe, barb, or foreign body was found. The crew did not apply a traction maneuver or attempt to reduce the shoulder.

Ellison accepted a cold pack positioned to avoid direct pressure on the contact sites. The left arm was supported in a position of comfort for transport. Medication options were discussed under the service protocol; Ellison elected to defer oral pain medication until hospital evaluation. No intravenous medication, sedative, or restraint was administered by the medical crew.

On reassessment, pulse was 92, respirations remained unlabored, and Ellison continued to speak coherently. She reported persistent shoulder soreness and reduced burning. Distal hand movement, sensation, and circulation remained intact. No new neurological complaint developed.

Cole discussed transport with Ellison using short explanations and pauses. Ellison agreed to go to Alder Creek Regional Medical Center. The crew coordinated with officers so the transition would not involve simultaneous commands. Any police handcuffs were managed by officers; the medical team did not use them as a clinical device. Ellison entered the ambulance with assistance and without another use of force observed by the crew.

Transport narrative and hospital handoff

Transport began at approximately 20:18. Iris Ellison followed separately after receiving destination information. During transit, Mara described shoulder pain as moderate with movement and mild at rest. She continued to report tenderness at the wrist dressing and two localized burning areas. She denied dizziness, nausea, new weakness, or trouble breathing.

Cole repeated vital observations and monitored cardiac rhythm because the dispatch involved a conducted-energy device. The rhythm remained regular, and no acute change occurred. The record does not imply that routine monitoring proves the force harmless; it states only the measured condition during transport.

At Alder Creek Regional Medical Center, Cole gave the receiving clinician a source-attributed handoff: welfare encounter; one drive-stun cycle reported by police; no probes; descent during restraint; left-wrist abrasion; two superficial contact marks; and left-shoulder pain. He also relayed the absence of reported loss of consciousness and Ellison's denial of a strike or equipment reach. Rusk's contrary description was noted as an officer account, not adopted as fact.

Ellison's property bag contained a phone and keys. No weapon was transferred with the patient. Care responsibility passed to the hospital. The ambulance cleared after completing the electronic chart and verifying that the destination received the clinical handoff.

Clinician certification and record limits

I, fictional Paramedic Mateo Cole, certify that this report accurately reproduces the simulated observations, care, and statements documented for the February 3 response. I distinguish direct clinical findings from histories supplied by Ellison, Rusk, Vega, dispatch, and Iris. I did not see the physical encounter and cannot determine whether the hidden arm movement was a strike, recoil, reach, or incidental contact.

The terms “abrasion,” “contact area,” and “shoulder pain” describe field observations, not final diagnoses. Hospital imaging and follow-up records provide later findings. A normal rhythm strip and stable transport do not answer the Fourth Amendment question. Likewise, a patient’s distress does not establish dangerousness or eliminate the need for safety assessment.

Times derive from the synthetic dispatch and patient-care systems and may reflect entry rather than first observation by a few seconds. The reported device duration comes from officer information available at handoff; the medical unit did not download the device. No field evidence suggested more than one cycle, and no probes were present.

This exhibit was generated for the fictional civil docket on November 21, 2025. It authenticates an invented clinical record only. Alder Creek Fire and Medical, its personnel, and the patient event do not correspond to any actual service or person, and the document supplies no medical or legal advice.